



Aug 30th 2010

DISCHARGE SUMMARY

Patients Name : Ms. Seema Agarwal

Age : 33 years

Hospital No : APJ1.0001193041

Consultant : Dr. S.V.S.S Prasad

Diagnosis : Metastatic Pancreatic Adeno Carcinoma with Bony metastasis.

History:

Admitted for week III Abraxane.

Present Status O/E:

Afebrile

B.P – 120/80 mm of hg

Heart Lungs /NAD

General condition stable

Discussion:

Received chemotherapy uneventfully

Treatment Received:

Tab. Apricap 125 mg

Inj. Gemcite 1600 mg IV

Inj. Abraxane 190 mg IV

IV Fluids

Antiemetics



Date of Commencement:

30/08/2010

Date of Completion:

30/08/2010

Condition of Discharge:

Stable

Advice on Discharge:

1. Tab. Zofer 8 mg thrice daily x 3 days (½ hour before food)
2. Tab. Aprecap 80 mg once daily on 31/8/10, 1/09/10.
3. Betadine gargles after feeds. (1/2 in tea spoon full ½ cup of water)
4. Cap. Riconia once daily (after lunch).
5. Home cooked food/Boiled water for drinking.
6. Inform emergency/ Dr. S.V.S.S Prasad if required.
7. Report CBP on 3/9/2010 and inform report to Dr. Prasad. , Serum CA 19-9
8. Tab. Taxceva 150 mg daily.
9. Inj. Clexane as advised by Dr. Manish.
10. Tab. Pantodac 40 mg daily.
11. Inj. Neupogen 300 mg S/C once daily on 31/8, 1/9.
12. Tab. Decadran 0.5 mg 2 tab thrice daily x 5 days.
13. Dupalac syrup 20ml sos in case of constipation.
14. Tab. Glaxo 500 mg daily x 5 days
15. CHERICOFF Syrup 2 tsf TDS


Dr. S.V.S.S Prasad



August 23rd 2010

DISCHARGE SUMMARY

Patients Name : Ms. Seema Agarwal

Age : 33 yearsS

Hospital No : APJ1.0001193041

Consultant : Dr. S.V.S.S Prasad

Diagnosis : Unknown primary with Bone and lymphnodemets.

History:

Admitted for cycle III Chemotherapy-D8S

Present Status O/E:

Afebrile

B.P – 120/80 mm of hg

Heart Lungs /NAD

General condition stable

Discussion:

Received chemotherapy uneventfully

Treatment Received:

Inj. Gemcite 1800 mg IV

Inj. Abraxane 190 mg IV

Tab.Apricap125 mg

IV Fluids

Antiemetics



Date of Commencement:

23/08/2010

Date of Completion:

23/08/2010

Condition of Discharge:

Stable

Advice on Discharge:

1. Tab. Zofer 8 mg thrice daily x 3 days ($\frac{1}{2}$ hour before food)
2. Tab. Aprecap 80 mg once daily on 24/8/10, 25/07/10.
3. Betadine gargles after feeds. (1/2 in tea spoon full $\frac{1}{2}$ cup of water)
4. Cap. Riconia once daily (after lunch).
5. Home cooked food/Boiled water for drinking.
6. Inform emergency/ Dr. S.V.S.S Prasad if required.
7. CBP check on 28/8/10 and inform reports to Dr.S.V.S.S.Prasad.
8. Tab. Tarceva 150 mg daily.
9. Inj. Clexane 60 mg s/c twice daily.
10. Mupirocin Cream for external application
11. Tab. Pantodac 40 mg daily
12. Review on 30/08/2010 for DAY 15 chemotherapy.
13. Inj. Neupogen 300mg s/c OD on 24/8, 25/8 at 8.00 p.m.
14. Diphthalac Syrup 20ml tid in case of constipation.
15. Tab ACTIFED 1 tab b.d.
- 0 - 0 - x 2 days.

[Signature]
Dr. S.V.S.S Prasad



DISCHARGE SUMMARY

Patients Name : Ms. Seema Agarwal

Age : 33 years

Hospital No : APJ1.0001193041

Consultant : Dr. S.V.S.S Prasad

Diagnosis : Unknown primary with Bone and lymphnodemets.

History:

Admitted for cycle III Chemotherapy. (1st with TAXANE)

Present Status O/E:

Afebrile

B.P – 120/80 mm of hg

Heart Lungs /NAD

General condition stable

Discussion:

Received chemotherapy uneventfully

Treatment Received:

Inj. Gemcite 1800 mg IV

Inj. Abraxane 190 mg IV

Tab.Apricap125 mg

IV Fluids

Antiemetics

Inj. Zometa 4mg IV



Date of Commencement:

16/08/2010

Date of Completion:

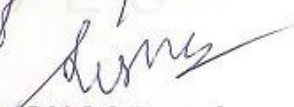
16/08/2010

Condition of Discharge:

Stable

Advice on Discharge:

1. Tab. Zofer 8 mg thrice daily x 3 days (½ hour before food)
2. Tab. Aprecap 80 mg once daily on 17/8/10, 18/07/10.
3. Betadine gargles after feeds.(1/2 in tea spoon full ½ cup of water)
4. Cap. Riconia once daily (after lunch).
5. Home cooked food/Boiled water for drinking.
6. Inform emergency/ Dr. S.V.S.S Prasad if required.
7. CBP check on 21/8/10 and inform reports to Dr.S.V.S.S.Prasad.
8. Tab. Tarceva 150 mg daily.
9. Inj. Clexane 60 mg s/c twice daily.
10. Mupirocin Cream for external application
11. Tab. Pantodac 40 mg daily
12. To see Dr. Manish on 17/08/2010 for changing Clexane to Acitron with PT/INR/APTT.
13. Review on 23/08/2010 for DAY8 chemotherapy.
14. Inj. Neupogen 300 µg qd OD on 17/8, 18/8 at 8.00 p.m.
15. Diphthalat Syrup 20 ml tds in case of constipation.


Dr. S.V.S.S Prasad



JULY 26th 2010

DISCHARGE SUMMARY

Patients Name : Ms. Seema Agarwal

Age : 33 years

Hospital No : APJ1.0001193041

Consultant : Dr. S.V.S.S Prasad

Diagnosis : Metastatic Pancreatic Adeno Carcinoma with Bony metastasis.

History:

Admitted for cycle II D8 Chemotherapy.

Present Status O/E:

Afebrile

B.P – 120/20 mm of hg

Heart Lungs /NAD

General condition stable

Discussion:

Received chemotherapy uneventfully

Treatment Received:

Inj. Gemcite 2000 mg IV

IV Fluids

Antiemetics

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Date of Commencement:

26/07/2010

Date of Completion:

26/07/2010

Condition of Discharge:

Stable

Advice on Discharge:

1. Tab. Zofer 8 mg thrice daily x 3 days (½ hour before food)
2. Tab. Aprecap 80 mg once daily on 27/7/10, 28/07/10.
3. Betadine gargles after feeds. (1/2 in tea spoon full ½ cup of water)
4. Cap. Riconia once daily (after lunch).
5. Home cooked food/Boiled water for drinking.
6. Inform emergency/ Dr. S.V.S.S Prasad if required.
7. Review on 09/08/2010 with CBP, S.Creatinine, and S.Bilirubin for DAY 8 chemotherapy.

8. Tab Tarceva 150 mg daily after breakfast

9. Duphalac 30 ml HS BD

10. Pentamyl patch

11. Inj Neupogen 300 µg qd OD on 27/7, 28/7 at 6.00 p.m.

12. Inj Clexane 60 mg qd BD

13. Tab Pantodac 40 mg OD before breakfast

14. Tab Lomtil 2 tab stat in case of diarrhoea and inform doctor.

Dr. S.V.S.S Prasad



DISCHARGE SUMMARY

Patients Name : Ms. Seema Agarwal

Age : 33 years

Hospital No : APJ1.0001193041

Consultant : Dr. S.V.S.S Prasad

Diagnosis : Metastatic Pancreatic Adeno Carcinoma with Bony metastasis.

History:

Admitted for cycle II D1 Chemotherapy.

Present Status O/E:

Afebrile

B.P – 120/20 mm of hg

Heart Lungs /NAD

General condition stable

Discussion:

Received chemotherapy uneventfully

Treatment Received:

Tab. Apricap 125 mg

Inj. Gemcite 2000 mg IV

Inj. DACOTIN 200 mg IV

Inj. Calcium gluconate 10ml IV

Inj. Magnesium sulphate 1Amp IV

Inj. ZOMETA 4 mg IV

IV Fluids

Antiemetics

Date of Commencement:

19/07/2010

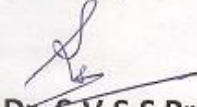
Date of Completion:

19/07/2010

Condition of Discharge:

Stable

Advice on Discharge:

1. Tab. Zofer 8 mg thrice daily x 3 days ($\frac{1}{2}$ hour before food)
 2. Tab. Aprecap 80 mg once daily on 20/7/10, 21/07/10.
 3. Betadine gargles after feeds. ($\frac{1}{2}$ tea spoon full $\frac{1}{2}$ cup of water)
 4. Cap. Riconia once daily (after lunch).
 5. Home cooked food/Boiled water for drinking.
 6. Inform emergency/ Dr. S.V.S.S Prasad if required.
 7. Review on 26/7/2010 with CBP, S.Creatinine, S.Bilirubin for DAY8 chemotherapy.
 8. Tab TARCEVA 150 mg daily after breakfast
 9. Diphthalae 30 ml HS (Thrice daily if necessary)
RTN
 10. Continuous Fentanyl patch
 11. Inj Neupogen 300mg s/c OD on 20/7 and 21/7 at 8:00 p.m. 
Dr. S.V.S.S Prasad
 12. Inj clexane 60 mg q/c BD
 13. Tab Pantodae 40 mg OD before breakfast
 14. Tab Lomtil 2 tab stat in case of diarrhoea and inform doctor.
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DISCHARGE SUMMARY

Patients Name : Ms. Seema Agarwal

Age : 33 years

Hospital No : APJ1.0001193041

Consultant : Dr. S.V.S.S Prasad

Diagnosis : Metastatic Pancreatic Adeno Carcinoma with Bony metastasis.

History:

Admitted for cycle I D8 Chemotherapy.

Present Status O/E:

Afebrile

B.P – 120/20 mm of hg

Heart Lungs /NAD

General condition stable

Discussion:

Received chemotherapy uneventfully

Treatment Received:

Tab. Apricap 125 mg

Inj. Gemcite 2000 mg IV

IV Fluids

Antiemetics



Date of Commencement:

02/07/2010

Date of Completion:

02/07/2010

Condition of Discharge:

Stable

Advice on Discharge:

1. Tab. Zofer 8 mg thrice daily x 3 days (½ hour before food)
2. Tab. Aprecap 80 mg once daily on 03/07, 04/07/2010.
3. Betadine gargles after feeds.
4. Cap. Riconia once daily (after lunch).
5. Home cooked food/Boiled water for drinking.
6. Inform emergency/ Dr. S.V.S.S Prasad if required.
7. Review on 16/7/2010 with CBP, S.Creatinine, S.Bilirubin *for cycle II - 5, Chemotherapy*
8. *Duphalac 15 ml HS x 3 days*
9. *Inj. NEUPROGEN 300mg q/c OD x 2 days*
10. *Check CBP on 7/7/2010, 11/7/2010 for Dr. S.V.S.S Prasad*
Inform report to Dr Prasad
11. *Inform immediately in case of excessive bleeding*

Dept. of Medical Oncology

Discharge Summary

Patient Details

UHID : APJ1.0001193041

IP No: IP15207

Name: Mrs. SEEMA AGARWAL

Age: 33Yr 2Mth 16Days

Sex: Female

Address: AGARWAL315-1-15-1-,560,SIDDIMBAR BAZAR,Hyderabad,Andhra Pradesh,India.

Bed Details: Regency Floor II , 9042 , BED No: 9042

Date of admission: 17-Jun-2010 12:29

Date of Discharge: 27-Jun-2010 02:16

Treating Doctors

Primary Consultant : Dr. PRASAD S V S S--Medical Oncology--MBBS,MD

(Paediatrics),DM (Medical Oncology & Haemoncology)

Consultants : Dr. MANISH DUGAR --Rheumatology--MBBS,MD (Internal
Medicine) ,FRACP(Rheumatology),

Dr. PRASAD S V S S--Medical Oncology--MBBS,MD

(Paediatrics),DM (Medical Oncology & Haemoncology),

Dr. S C SAMAL~ Dr K A RAMAKRISHNA--Gastroenterology

Surgeons/Consultants :Dr. PRASAD S V S S--Medical Oncology--MBBS,MD

(Paediatrics),DM (Medical Oncology & Haemoncology)

Anesthesiologists : Dr. SANATH REDDY P--Anaesthesiology--MBBS,MD

(Anaesthesia)

Diagnosis

Disease Type

Disease Name

Pd

METASTATIC PANCREATIC ADENOCARCINOMA WITH BONY
METASTASIS, DVT AND PULMONARY EMBOLISM.

Pd

SEVERE VITAMIN D DEFICIENCY

Surgery/Procedure

Surgery Name

BONE MARROW BIOPSY

Date of Surgery/Procedure

22-Jun-2010

Chief Complaint(s) and Present Known Illness

Chief Complaint(s) : Pain in Calves for 6 weeks.

Present Known Illness : Mrs. Seema was admitted for evaluation of Venous doppler proven deep vein thrombosis of both lower limbs and painful nodules over her forearms and left foot.

Patient History

Family History : Mother - breast cancer.

History of Drug Allergy : No Known drug allergies

Physical Examination

General Examination :

Height : 5 ft	Weight : 90.00Kg	BP : 120 / 80 mmHg
Pulse : 78 /min	Temperature : 99 F	Respiration : 20 /min
Pallor : No	Cyanosis : No	Icterus : No
Clubbing : No	Edema : No	Lymphadenopathy : No

Systemic Examination:

Head & Neck : NAD	CVS : NAD
Central Nervous System : NAD	Respiratory : NAD
Genito urinary : NAD	Gynaecological : NAD
Gastro Intestinal : No palpable mass abdomen.	ENT : NAD
Skin : NAD	Eye & Fundus : NAD
Spine & Joints : - Tender nodules right forearm, left foot. - Superficial thrombophlebitis left forearm and elbow. - No breast mass except a vague lump left upper quadrant (Fibroadenosis). Cervical and axillary Lymphadenopathy.	Dental : NAD
Others : NAD	

Investigation Reports

BIOCHEMISTRY

ALBUMIN - SERUM

ALBUMIN - SERUM	5.0 g/dL
GLOBULIN - SERUM:(Calculated)	5.3 g/dL
ALBUMIN:GLOBULIN - SERUM(RATIO) (Calculated)	0.9
ALBUMIN - SERUM	3.5 g/dL

ALKALINE PHOSPHATASE - SERUM/PLASMA

ALKALINE PHOSPHATASE - SERUM/PLASMA	251 U/L
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ALT(SGPT) - SERUM / PLASMA

ALT(SGPT) - SERUM / PLASMA	50 U/L
ALT(SGPT) - SERUM / PLASMA	50 mg/dL

AST (SGOT) - SERUM

AST (SGOT) - SERUM	44 U/L
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BILIRUBIN CONJUGATED (DIRECT) - SERUM

BILIRUBIN CONJUGATED (DIRECT) - SERUM	0.1 mg/dL
BILIRUBIN UNCONJUGATED - SERUM(Calculated)	0.5 mg/dL

BILIRUBIN, TOTAL - SERUM

BILIRUBIN, TOTAL - SERUM	0.6 mg/dL
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BLOOD SUGAR(FASTING)-(LIFE STYLE-ATFL 2002/IC-ITC)

BLOOD SUGAR(FASTING)-(LIFE STYLE-ATFL 2002/IC-ITC)	100
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CA (CANCER ANTIGEN) 125 - SERUM

CA (CANCER ANTIGEN) 125 - SERUM	361 U/mL
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CA (CANCER ANTIGEN) 19-9 - SERUM

CA (CANCER ANTIGEN) 19-9 - SERUM	30200 U/mL
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CALCIUM - SERUM

CALCIUM - SERUM	9.4 mg/dL
CALCIUM - SERUM	8.7 mg/dL

CARBONDIOXIDE, TOTAL - SERUM/PLASMA

CARBONDIOXIDE, TOTAL - SERUM/PLASMA	26 mmol/L
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CEA: CARCINO EMBRYONIC ANTIGEN - SERUM

CEA: CARCINO EMBRYONIC ANTIGEN - SERUM	5.19 ng/mL
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CHLORIDE - SERUM / PLASMA

CHLORIDE - SERUM / PLASMA	94 mmol/L
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CHOLESTEROL - SERUM / PLASMA

CHOLESTEROL - SERUM / PLASMA	217 mg/dL
CHOLESTEROL - SERUM / PLASMA	186 mg/dL

CREATININE - SERUM / PLASMA

CREATININE - SERUM / PLASMA	0.9 mg/dL
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GGTP: GAMMA GLUTAMYL TRANSPEPTIDASE - SERUM

GGTP: GAMMA GLUTAMYL TRANSPEPTIDASE - SERUM	74 U/L
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HDL CHOLESTEROL - SERUM / PLASMA

HDL CHOLESTEROL - SERUM / PLASMA	41 mg/dL
HDL CHOLESTEROL - SERUM / PLASMA	38 mg/dL

HOMOCYSTEINE - SERUM / PLASMA

HOMOCYSTEINE - SERUM / PLASMA	12.7 µmol/L
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PHOSPHORUS, INORGANIC - SERUM

PHOSPHORUS, INORGANIC - SERUM	4.0 mg/dL
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POTASSIUM - SERUM / PLASMA

POTASSIUM - SERUM / PLASMA	4.1 mmol/L
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PROTEIN ELECTROPHORESIS - 24 HR URINE

Total Urinary Volume(24 hrs):	2100 mL/day
PROTEIN, TOTAL - 24 HR URINE	42 mg/day
INTERPRETATION:	THE ELECTROPHOREGRAM SHOWED NO BANDS.
Impression	SUGGESTIVE OF NORMAL PATTERN- WITH NO DETECTABLE LOSS OF PROTEIN FRACTIONS.

PROTEIN ELECTROPHORESIS - SERUM

Albumin Fraction:	Lowered.
Alpha-1 Fraction:	Raised.
Alpha-2 Fraction:	Raised.
Beta-1 Fraction:	With In Normal Limits.
Beta-2 Fraction:	Raised.
Gamma Fraction:	With In Normal Limits.
A/G Ratio:	Suppressed.

PROTEIN, TOTAL - SERUM / PLASMA

PROTEIN, TOTAL - SERUM / PLASMA	10.3 g/dL
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PROTEIN,TOTAL - SPOT URINE

PROTEIN,TOTAL - SPOT URINE	6 mg/dL
CREATININE - SPOT URINE(Modified Jaffe:Alkaline Picrate)	56 mg/dL

SODIUM - SERUM / PLASMA

SODIUM - SERUM / PLASMA	134 mmol/L
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TRIGLYCERIDES - SERUM

TRIGLYCERIDES - SERUM	132 mg/dL
LDL CHOLESTEROL - SERUM / PLASMA: (Calculated)	150 mg/dL
TRIGLYCERIDES - SERUM	193 mg/dL
LDL CHOLESTEROL - SERUM / PLASMA: (Calculated)	109 mg/dL

UREA - SERUM / PLASMA

UREA - SERUM / PLASMA	19 mg/dL
UREA NITROGEN SERUM/PLASMA:(Calculated)	9 mg/dL

VITAMIN B12 -SERUM

VITAMIN B12 - SERUM	>2000 pg/mL
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VITAMIN D3(25-OH)-SERUM/PLASMA

VITAMIN D3(25-OH)-SERUM/PLASMA	<4.0 ng/mL
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VITAMIN FOLATE - SERUM

VITAMIN FOLATE - SERUM	11.6 ng/mL
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HAEMATOLOGY**ACTIVATED PARTIAL THROMBOPLASTIN TIME**

APTT	28.9 sec.
Control	30

BONE MARROW CORE BIOPSY ONLY

Brief clinical history	Multiple osteolytic, lesions with DVT ? Mets/? myeloma/? TB.
Macroscopic Examination:	Multiple ,linear brown bits each measuring 0.5 cms long ,-ETP.
Microscopic description:	Trephine core sections showing mostly marrow clots and tiny trephine cores showing foci of normo cellular marrow with normal hemotopoiesis. Rest of the marrow clot section and foci of trephine core showing infiltration with large anaplastic epithelium in clusters, sheets, and discret cells and some areas with gland formation.
Conclusion:	The BMB is consistent with that of infiltration with metastatic adenocarcinoma deposits.

BONE MARROW SMEAR/SLIDE FOR OPINION

Brief clinical history	Recently diagnosed C/O DVT ? MM / ? Lymphoma/ ? TB / ? Mets
Blood Smear:	Leukocytosis +, with shift to right, megathrombocytes +, micro spherocytes +
Bone Marrow Aspiration:	Marrow aspirated from left post, iliac crest Marrow is sparsely cellular Megakaryocytes are present, appear normal Erythroid : Myeloid ratio is 1.2:1. Erythropoiesis is normoblastic Granulopoiesis is relatively diminished Myelogram showed predominance of normoblasts and a few discrete, atypical epithelial cells, clusters with intracytoplasmic ? mucin vacuoles. Myelogram : Neutrophils : 18, Normoblasts : 62, Blasts : 01, Myelocytes : 08, Metamyelocytes : 04, Band forms : 03, Abnormal cells : 04 %, Background showed infiltration with discrete & clusters of atypical epithelial cells, at areas forming gland like structures.

Conclusion:	The BMA/BMI cytology is consistent with that of infiltration with metastatic adenocarcinoma deposits.
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C B C WITH ESR (AUTOMATION + STANDARD METHOD)

Hemoglobin	13.9 g/dl
Haematocrit	43.6 %
RBC COUNT	4.97 Million/ul
MCV	88 fl
MCH	27.9 pg
MCHC	31.9 g/dl
RDW	11.2 %
TLC Count	13.3 10 ³ /mm ³
Platelet Count	233 10 ³ /mm ³
ERYTHROCYTE SEDIMENTATION RATE (ESR)	38 mm/1st hr
Neutrophils	76 %
Lymphocytes	18 %
Eosinophils	04 %
Monocytes	02 %
Basophils	00 %
RBC:	Normocytic & normochromic
WBC:	Neutrophilic leukocytosis +, eosinophilia +
Platelets:	Adequate on the smear

ERYTHROCYTE SEDIMENTATION RATE (ESR)

ERYTHROCYTE SEDIMENTATION RATE (ESR)	25 mm/1st hr
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PROTHROMBIN TIME

Prothrombin Time (PT): Test	17.6 sec.
Mean Normal Prothrombin Time (MNPT)	13.5 sec.
INR	1.42
Prothrombin Time (PT): Test	34.1 sec.
Mean Normal Prothrombin Time (MNPT)	13.5 sec.
INR	3.37

SCREENING FOR LUPUS ANTI-COAGULANT)(dRVVT)

SCREENING FOR LUPUS ANTI-COAGULANT) (dRVVT)	Please repeat the test after six weeks.
SCREENING FOR LUPUS ANTI-COAGULANT) (dRVVT)	Positive

URINE- BENCE JONES PROTEINS

URINE- BENCE JONES PROTEINS	Negative
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URINE ROUTINE (CUE)

Specific Gravity	1.010
Colour:	Pale-Yellow
Transparency:	Clear

Reaction:	Acidic
Protein :	Nil
Glucose:	Nil
pH	5
Blood:	Negative
Ketone	Negative
Bile Pigments:	Negative
Bile Salts(Hay's Test):	Negative
Urobilinogen	Normal E.U./dL
Nitrite	Negative
WBC/Pus Cells	2-4 /hpf
Tc/Sqc(Transitional/Squamous epithelial cells)	4-6 /hpf
Specific Gravity	1.005
Colour:	Pale-Yellow
Transparency:	Clear
Reaction:	Acidic
Protein :	Negative
Glucose:	Negative
pH	6.5
Blood:	Negative
Ketone	Negative
Bile Pigments:	Negative
Bile Salts(Hay's Test):	Negative
Urobilinogen	Normal E.U./dL
Nitrite	Negative
WBC/Pus Cells	2-3 /hpf
Tc/Sqc(Transitional/Squamous epithelial cells)	2-3 /hpf

HISTOPATHOLOGY

IHC - PACKAGE - 1 (5 TO 8 ANTIGENS)

ACTH:	
AFP	
ALK:	
AMHCR(p504s)	
Alphafetoprotein:	
Amyloid AA:	
B Catenin	
Bcl 2:	
Bcl 6:	
Brief Clinical History:	
C4d:	
CD 10:	
CD 15:	
CD 20:	
CD 23:	
CD 30:	

CD 31:	
CD 34:	
CD 3:	
CD 45:	
CD 5	
CD 56:	
CD 57:	
CD 68:	
CD 79a:	
CD 99:	
CD-117:	
CD-138:	
CDX2:	Negative.
CEA:	
CK - HMW:	
CK 10	
CK 19:	Positive.
CK 20:	Negative.
CK 5/6:	
CK 7	Strong cytoplasmic positive.
CK 8/18 :	
CMV:	
Calcitonin :	
Caldesmon:	
Calretinin:	
Chromogranin:	
Cyclin D1:	
Cytokeratin - Pan (AE1/AE3) :	
Desmin :	
Diagnosis/Comments:	The biopsy shows features of a metastatic poorly differentiated carcinoma with focal intracytoplasmic mucin (confirmed on special stain). The immune profile, positivity for CK7 and CK19 and negativity for the other markers mentioned above is in favour of tumour spread from a primary pancreatobiliary carcinoma.
E.Cadherin:	
EMA:	
Estrogen receptor:	Negative.
FSH:	
GCDFP-15:	
GFAP:	
GH:	
Galectin 3:	
Growth Hormone:	

H M W C K	
HBME-1:	
HMB 45:	
Hepatocyte:	
Her 2 neu:	Score - 0/3 - Negative.
Hormones-Pituitary:	
IgA:	
Inhibin:	
JCBK	
Kappa Lc:	
LH:	
Lamda Lc:	
Leucocyte Common Antigen:	
MPO	
Mib 1 (Ki 67):	
Myeloperoxidase:	
Myogenin:	
NSE:	
Neuron Specific Enolase:	
P 16:	
P 53:	
PAX - 5	
PLAP:	
PSA:	
Pancytokeratin:	
Progesterone Receptor:	Negative.
Prolactin:	
Ref No:	BMB-188/10.
S-100 Protein:	
Smooth Muscle Actin:	
Specimen:	Bone marrow biopsy (IHC-610-10).
Synaptophysin:	
TTF(Thyroid Transcription Factor):	Negative.
Tdt:	
Thyroglobulin:	
Vimentin:	

MICROBIOLOGY

ACID FAST STAIN [BODY FLUID]

AFB Smear	No AFB seen
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AFB CULTURE [ICD FLUID](Bact Alert 3D)

Smear Findings	No AFB seen
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COMPLEMENT LEVEL - C3

COMPLEMENT LEVEL - C3/C3c	1.63 g/l
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COMPLEMENT LEVEL C4

COMPLEMENT LEVEL C4	0.426 g/l
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C-REACTIVE PROTEIN (CRP)

C-REACTIVE PROTEIN (CRP)	84.4
C-REACTIVE PROTEIN (CRP)	84.4 mg/L
C-REACTIVE PROTEIN (CRP)	Positive
C-REACTIVE PROTEIN (CRP)	Positive mg/L
Comments :	<3 is Negative
C-REACTIVE PROTEIN (CRP)	13.5
C-REACTIVE PROTEIN (CRP)	13.5 mg/L
C-REACTIVE PROTEIN (CRP)	Positive
C-REACTIVE PROTEIN (CRP)	Positive mg/L
Comments :	<3 is Negative

H I V - 1+2 SCREENING(Ag+Ab)

H I V - 1+2 SCREENING(Ag+Ab)	Non-Reactive
INTERPRETATION :	NEGATIVE

HBsAg

HBsAg	NEGATIVE
HBsAg	NEGATIVE S/N

IMMUNOGLOBULIN - IgA

IMMUNOGLOBULIN - IgA	3.26 g/L g/l
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IMMUNOGLOBULIN - IgG

IMMUNOGLOBULIN - IgG	15.6 g/L g/l
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IMMUNOGLOBULIN - IgM

IMMUNOGLOBULIN - IgM	1.53 g/L g/l
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PROCALCITONIN SCREENING

Procalcitonin Screening level:	0.08
Procalcitonin Screening level:	0.08 ng/ml

RA FACTOR

RA FACTOR	NEGATIVE
RA FACTOR	NEGATIVE mg/L

VDRL (QUALITATIVE) - BLOOD

VDRL (QUALITATIVE) - BLOOD	Non-Reactive
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TRANSPLANTATION IMMUNOLOGY**ANTI B2-GLYCOPROTEIN-1(IgG)**

Dilution:	1:200
RESULT:	NEGATIVE
Detected levels:	04

ANTI CARDIOLIPIN Ab IgG

Detected levels:	04
RESULT:	NEGATIVE

ANTI CCP (CYCLIC CITRULLINATED PEPTIDE)

Dilution:	1:100
RESULT:	NEGATIVE
Detected levels:	01

ANTI NEUTROPHIL CYTOPLASM ANTIBODIES-(ANCA-IgG)

Dilution:	1:10
RESULT:	c-ANCA & p-ANCA=NEGATIVE

ANTI NUCLEAR ANTIBODIES-IgG

Dilution:	1:100
RESULT:	NEGATIVE
Reaction Pattern:	Nil
Suspected Antigen Specificity:	None

C T SCAN RADIOLOGY**MDCT SCAN CHEST (WITH CONTRAST)**

MDCT sections of chest were studied with contrast.

Bilateral axillary lymphnodes noted.

MEDIASTINUM :

Soft tissue density stranding with nodularity noted in prevascular, aortopulmonary and precarinal areas.

Peritracheal soft tissue thickening noted.

Vessels appear normal.

Hypodense area in right upper and lower lobe segmental pulmonary arteries with peripheral contrast flow - suggestive of thrombus/ emboli.

LUNGS :

Soft tissue density area in both lower lobe posterior segment, right middle lobe lateral segment and lingula with subtle air bronchogram and adjacent fissural and pleural adhesions - suggestive of consolidation with ? early cavitation.

Bones show lytic areas.

Impression : 1. Soft tissue density stranding with nodularity noted in prevascular, aortopulmonary and precarinal areas - mediastinal lymphadenopathy.

2. Hypodense area in right upper and lower lobe segmental pulmonary arteries with peripheral contrast flow - suggestive of thrombus/ emboli.

3. Soft tissue density area in both lower lobe posterior segment, right middle lobe lateral segment and lingula with subtle air bronchogram and adjacent fissural and pleural adhesions - suggestive of consolidation with ? early cavitation.

Rule-out connective tissue disorder.

4. Bones show lytic areas - myeloma.

Correlate clinically and follow up.

COLOUR DOPPLER

VENOUS DOPPLER OF - BOTH LOWER LIMBS AND LEFT UPPER ARM

Suboptimal study.

Spontaneous flow seen in common femoral veins and superficial femoral veins on either sides.

Sluggish flow seen in right lower superficial femoral vein ? Chronic thrombosis with partial recanalisation.

Popliteal veins flow appear normal on either sides.

Anterior and posterior tibial veins on both sides are either side show sluggish and intermittent flow.

LEFT UPPER LIMB :

Flow in cephalic veins appear normal.

The basilic vein appears dilated with hypoechoic thrombus upto the levels of axilla.

The superficial veins of the ventral and dorsal aspect of forearm appear non compressible - suggestive thrombosis.

Impression :

FLUORO RADIOLOGY

FLURO-GUIDED FNAC

Impression :

RADIOLOGY

100ML CONTRAST(OMINIPAQUE) - C.T.SCAN

Impression :

ULTRASOUND RADIOLOGY

ULTRASOUND - BREAST

High resolution ultrasound of both breasts was performed in axial, radial and sagittal planes.

Skin, areola, nipple and subcutaneous fat on both sides are normal.

Both breasts are predominantly fatty, and do not show any obvious cysts or mass lesions.

Multiple small lymph nodes seen in both axillary regions, measuring < 12 mm each, showing loss of fatty central hilum - ? inflammatory.

Impression : No obvious breast parenchymal lesions.

Multiple small lymph nodes in both axillary region, measuring < 12 mm each, showing loss of fatty central hilum - ? inflammatory.

Sugg: Mammography, if clinically indicated.

ULTRASOUND - WHOLE ABDOMEN

LIVER : Measure (146 mm) normal in size . Echotexture is uniform and normal.
No I.H.B.D./ focal lesions.

PORTAL VEIN : Normal in size.

GALL BLADDER : Well distended with normal walls and lumen.
No calculi/pericholecystic fluid.

COMMON BILE DUCT : Normal in size.

PANCREAS : Normal in size and echotexture.

AORTA, IVC and para aortic areas are normal.

SPLEEN : Normal in size (92 mm) and echotexture.

KIDNEYS : Both kidneys are normal in site, size and echotexture.
No evidence of calculi/ hydronephrosis.
Right kidney measures : 91 x 42 mm
Left kidney measures : 95 x 47 mm

URINARY BLADDER : Well distended with normal walls.
No evidence of calculi/ growth/ diverticula .

UTERUS : Normal in size and echotexture.
Uterus measures : 82 x 39 x 47 mm
Combined endometrial thickness is 4 mm thickness.

OVARIES : Not visualised

OTHERS : No evidence of any free fluid in P.O.D.
Impression : - Normal study

X RAY

X-RAY BOTH FEMUR AP AND LAT

Bone density and trabecular pattern are normal.

No lytic or sclerotic lesions noted.

No evidence of any fracture seen.

Soft tissues are normal.

Impression : Normal study.

X-RAY DORSOLUMBAR SPINE - AP/LAT

Bone density is normal.

Alignment of the spine is normal.

Intervertebral disc spaces are normal.

The vertebral bodies, pedicles and neural arches are normal.

No evidence of sclerotic or lytic lesions seen.

No paraspinal soft tissue abnormality is seen.

Impression : Normal study.

X-RAY PELVIS AP

Contrast noted in the sigmoid colon from previous study.

Bone density and trabecular pattern are normal.

No evidence of any lytic or sclerotic lesion noted.

Both S.I. joints and hip joints are normal.

Soft tissues are normal.

Impression : 1. Both hip joints and S.I. joints are normal.

2. Contrast noted in the sigmoid colon from previous study.

X-RAY SKULL LAT

Evidence of calcific opacities in the frontal region adjacent to inner table.

Skull vault and sinuses are normal.

Sella tunica within normal limits.

Meningeal vascular pattern normal.

Irregular calcification noted in the soft tissue neck posteriorly.

Impression : 1) Calcific opacities in the frontal region adjacent to inner table.

2) Irregular calcification noted in the soft tissue neck posteriorly.

E C G

BED SIDE ECG

Ventricular Rate	97 / MIN
Q R S	0.09 SEC
QTc	0.44 SEC
Atria-Rate	97 / MIN
Rhythm	SR

Axis	+32°
P R Interval	0.15 SEC
P Wave	NORMAL
Voltage	NORMAL
Q Wave	NIL
St-T Changes	T III, V1 - V4
Final Impression	NON SPECIFIC 'T' WAVE CHANGES

ECHO LAB

2D-ECHO WITH COLOUR DOPPLER

Echo No.	287
Mitral Valve AML	Normal
PML	Normal
Aortic Valve	Normal
Tricuspid Valve	NORMAL
Plum Valve	NO PAH
Great Vessel Relationship	NORMAL
Left Atrium	NORMAL
Inter Atrial Spetum	INTACT
Inter Ventricular Spetum	
Intact/VSD	INTACT
IVS Thickness	0.7 CM
LV Post Wall	
Thickness	0.8 CM
Pericardium :	Normal
Dimensions :	
Aortic Root Diameter(ed)	3.2 CM
LA Dimension(ed)	3.2 CM
RAID (ed)	-
RVID (ed)	-
LVID (ed)	4.8 CM
LVID (es)	3.3 CM
Left Ventricular Function	
LVESVI	-
LVEDVI	-
%FD	30%
EF	0.59
Doppler Study	-

Final Diagnosis	ALL VALVES AND CHAMBERS NORMAL NO REGIONAL WALL MOTION ABNORMALITY INTACAT SEPTAE NORMAL LV FUNCTION NO PERICARDIAL EFFUSION/NO CLOT NO MR/AR/PR/TR.
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Summary

- Inj. Gemite 2000 mg once daily.
- Inj. Oxaliplatin (Dacotin) 200 mg once daily.
- Inj. Zometa 4 mg.
- I.V fluids.
- Antiemetics.
- Inj. Magnesium Sulphate and Calcium Gluconate with Chemotherapy.
- Analgesics.
- Inj. Clexane 60 mg Sc.
- Tab. Pantocid.
- Tab. Thyronorm.
- Fentanyl patch 12.5 mcg daily.

Discussion

Mrs. SEEMA AGARWAL, 33 years female was evaluated for her hypercoagulable state. CTPA showed pulmonary embolism and lytic lesions on thoracic vertebrae. A F-18 PET bone scan confirmed multiple bone lytic lesions. A whole body FDG PET scan showed increased septate in lymph nodes, left ovary, skeletal bones and a bulky pancreas. A work up for malignancy was done Ca - 19 - 9 was significantly elevated and BM biopsy showed adenocarcinoma. Immunohistochemistry showed positively for Ck 7 and Ck 19 suggesting a primary from Pancreatico-biliary system. She was given the 1st cycle of Palliative Chemotherapy and discharged home. The treatment plan and prognosis has been extensively discussed with the family.

Discharge Medication List

Prescription	Comments
PANTODAC 40 MG TAB(PANTOPRAZOLE) 1 Tablet 24 Hourly Before Breakfast	Once daily before breakfast to continue till next review
CLEXANE INJ - 60MG/0.6ML(ENOXAPARIN INJ) 1 Pre Filled Syringe 12 Hourly Not Applicable to Meals	Twice daily to continue till next review
DUPHALAC 200 ML SYP(LACTULOSE) 15 MI 24 Hourly After dinner	Once daily at bed time to continue till next review
BETADINE GARGLE - 100ML(POVIDONE IODINE GARGLE) 1 MI 24 Hourly Not Applicable to Meals	Imp. Gargle on every feed
DUROGESIC 25MG/H PATCHES 1*3S(FENTANYL) 1 Patches 24 Hourly Not Applicable to Meals	1/2 patch - To be changed on every 3rd day
NEUPOGEN 30 MIO (INDIAN)(FILGRASTIM) 1 Pre Filled Syringe 24 Hourly Not Applicable to Meals	Once daily on 27/06/2010 and 28/06/2010
DOLO-650MG(PARACETAMOL) 1 Tablet SOS Not Applicable to Meals	As and when required if there is fever / pain (SOS)
THYRONORM 50MCG TAB(THYROXINE) 1 Tablet 24 Hourly Before Breakfast	Once daily BBF to continue till next review
ZOFER 8 MG TAB(ONDANSETRON) 1 Tablet 8 Hourly Before Meal	Thrice daily before food for 2 days
SHELCOL 500MG TAB(ALFACALCIDOL) 1 Tablet 24 Hourly After Meal	Once daily at bed time to continue till further advice.
CALCIROL GRANULES(ALFACALCIDOL) 1 Sachets 24 Hourly Not Applicable to Meals	(60.000 thousand units of Vit-D3)for 6 weeks (Every SUNDAYS only)

Condition on Discharge

STABLE

Ambulatory	YES	Vitals Stable	Yes
Blood Sugar Levels - Controlled	Yes	Surgical Wound Clean	Yes
Pain Score Below 4	Yes		

Special Instructions

Dietician Advice :

Post Discharge Investigations :

Diet :HOME COOKED FOOD, BOILED WATER FOR DRINKING.

Follow-up Instructions :CBP, PT, INR ON 1/7/2010.

REVIEW WITH Dr. S.V.S.S PRASAD ON 2/7/2010.

Physical Activity :As tolerated.

Normal Activities can be resumed after :

Review Consultant :

Contact Information :

Dr. PRASAD S V S S

Primary Consultant

JrConsultant/Registrar/Resident

In case of bleeding from wound, temperature above 101 degree F, fresh pain and any other concern please contact Emergency Room Physician, Apollo Hospitals at 040-60601066 for routine appointment contact the hospital at 040-60601066.